22 YOF w/ Panic Attack (Chest Pain / Hyperventilation)

Scenario Set Up	**Equipment needed:** Pulse oximeter, BP cuff, stethoscope, glucometer, O2 (should NOT be needed), blanket. **PROCTOR:** You are a 22-year-old female college student sitting on the floor of a campus library bathroom. You are breathing fast, trembling, gripping your chest, and convinced you are having a heart attack or dying. You answer questions in short bursts between breaths and keep asking "Am I going to die?" and "Is it my heart?" You have never had an episode like this before and you are terrified. You have a midterm exam in one hour and have been awake most of the night studying with several energy drinks. A friend found you and called 911. If the EMTs are calm, make eye contact, explain what they are doing, and coach your breathing, you slowly improve over 3-5 minutes. If they are rushed, dismissive, or say "just calm down," you get worse (tingling spreads, you feel like you are going to pass out). Do NOT improve until they have at least obtained a full set of vitals and a BGL - they should not assume this is "just anxiety" without ruling out other causes.
Dispatch	Respond Code 3 to a 22-year-old female, chest pain and difficulty breathing, conscious and breathing, at the university library second-floor women's restroom. Caller is a friend on scene.
Scene Size Up	Scene is safe. Pt is sitting on the floor with her back against the wall, knees drawn up, hands shaking, breathing rapidly. One friend is kneeling next to her, visibly worried. No trauma, no vomit, no drug paraphernalia. Pt is crying and says she can't breathe. No obvious life threats.
Pertinent Primary Assessment Findings	**AVPU:** Alert, A&O x4 (person, place, time, event) - although distracted and hard to keep on topic. **A:** Patent, speaking in short sentences. **B:** Rapid and shallow, ~30/min, regular, no wheezes or stridor, lung sounds clear and equal bilaterally. Pt feels like she "can't get a full breath." **C:** Radial pulse strong, rapid, and regular. Skin pale, cool, and diaphoretic. Cap refill <2 sec.
Pertinent Secondary Assessment Findings	**OPQRST (chest pain):** O - sudden, started ~15 min ago while studying. P - nothing makes it better; worse when she thinks about the exam. Q - "tight," "like a band" across chest, also sharp with deep breaths. R - no radiation to arm/jaw/back. S - 7/10. T - constant since onset, not worsening. **Other complaints:** Tingling/numbness in both hands and around the mouth, dizziness, feeling of "unreality," sense of impending doom, nausea, chills. **SAMPLE** A - NKDA M - Birth control pill only. No inhaler, no cardiac meds. P - No history. Denies asthma, cardiac history, diabetes, prior anxiety diagnosis. Family history: mother has "anxiety." Denies drug use. Denies pregnancy (LMP 2 weeks ago). L - Energy drinks (3 cans overnight), coffee this morning, no solid food since dinner last night. E - Awake all night studying. Walked to the library, felt her heart "skip," then sudden onset of chest tightness and shortness of breath. Went to the bathroom to splash water on her face and it got worse.

Vitals	**Set 1 (initial):** BP 138/88, HR 124 regular, RR 30 shallow, BGL 96 mg/dL, SpO2 99% RA, skin pale/cool/moist, pupils PERRL, T 98.4 F. **Set 2 (after ~5 min of coached breathing & reassurance):** BP 126/80, HR 98, RR 18, SpO2 99% RA, skin pink/warm/dry, tingling resolved.
Treatments	**Necessary:** - Scene safety / BSI. Approach calmly; one EMT takes the lead and speaks slowly. - Full primary assessment and complete set of vitals including BGL, SpO2, and lung sounds (rule out hypoxia, hypoglycemia, asthma, cardiac cause, drug ingestion). - Position of comfort (sitting upright). - Coached breathing: have pt breathe with you, slow count in through nose / out through mouth. Do NOT have the pt breathe into a paper bag. - Reassurance in plain language: explain what you are doing, acknowledge that symptoms are real and frightening, and tell her that her vitals/oxygen are good. - Reduce stimulation: move the friend back if unhelpful, dim/quiet environment, remove bystanders. - Reassess vitals after intervention. - Transport for evaluation (first episode, chest pain with no prior history - needs an ECG and a physician evaluation to confirm). If pt refuses, obtain a proper refusal only after capacity is established. **Helpful but not life-saving:** Blanket for chills, keep friend close if she is calming, let pt hold something/plant feet on floor (grounding), 12-lead ECG if ALS is available.
Key Points	- Panic attack is a diagnosis of EXCLUSION in the field. EMTs must rule out cardiac, respiratory, metabolic, and toxicologic causes before attributing symptoms to anxiety. The caffeine load and sleep deprivation are contributing factors but do not change the workup. - Carpopedal spasm, perioral tingling, and dizziness result from hyperventilation blowing off CO2 (respiratory alkalosis) - this is NOT hypoxia; SpO2 stays normal. Supplemental O2 is not indicated with SpO2 99% and will not help. - Never tell the patient to "calm down" or imply she is faking. Validation + a calm provider are the main treatment. - Paper-bag rebreathing is no longer recommended (risk if the cause is actually hypoxic). - Symptoms typically peak within 10 minutes and resolve within 20-30 minutes. - If this were a known panic-disorder pt with identical prior episodes and normal vitals, a refusal might be reasonable; for a FIRST episode with chest pain, transport should be strongly encouraged.

Bonus Questions	1. Why does the pt have tingling around her mouth and in her hands? (A: Hyperventilation lowers CO2, causing respiratory alkalosis; this causes vasoconstriction and neurological symptoms such as paresthesia and carpopedal spasm.) 2. Name four life-threatening conditions that can mimic a panic attack. (A: MI/ACS, pulmonary embolism, asthma, hypoglycemia, dysrhythmia, stimulant/sympathomimetic overdose, hyperthyroid crisis.) 3. What is the difference between a panic attack and panic disorder? (A: A panic attack is a single episode; panic disorder is recurrent unexpected attacks with persistent worry/behavior change for a month or more.) 4. Why is a paper bag not recommended? (A: It can cause hypoxia if the underlying problem is cardiac or respiratory, and it reinforces the idea that the pt is doing something wrong.) 5. What finding in this scenario would make you upgrade your concern for a cardiac cause? (A: Pain radiating to arm/jaw, exertional onset, irregular pulse, SpO2 drop, no improvement with coaching, abnormal 12-lead.)

34 YOM w/ Acute Psychosis (Schizophrenia, Auditory Hallucinations)

Scenario Set Up	**Equipment needed:** BP cuff, stethoscope, pulse oximeter, glucometer, stretcher with straps. (Restraints should NOT be needed if the scenario is run well.) **PROCTOR #1 (patient):** You are a 34-year-old male who lives alone in a small apartment. You were diagnosed with schizophrenia at 24 and were stable on medication until you stopped taking it about three weeks ago because you believed the pills were "tracking devices." You are hearing two voices - one that comments on everything the EMTs do and one that tells you the EMTs are "with them." You occasionally look over the EMTs' shoulders and respond to the voices out loud ("I know, I KNOW"). You believe your neighbor has put cameras in your vents. You are suspicious and guarded but NOT violent. You pace and do not like being cornered or touched without warning. If the EMTs are honest, ask permission before touching you, keep their voices low, don't argue about whether the cameras are real, and give you simple choices, you gradually cooperate and agree to go to the hospital. If they lie to you, crowd you, laugh, or try to "prove" the voices aren't real, you become louder, refuse all care, and start demanding they leave. **PROCTOR #2 (sister):** You called 911 because you came to check on your brother and found the windows covered in foil and him talking to the walls. You can provide SAMPLE history. You are worried, not angry.
Dispatch	Respond Code 2 to a 34-year-old male, psychiatric emergency, acting strangely. Caller is the patient's sister. PD has been requested and is en route; ETA unknown.
Scene Size Up	Apartment door is open; sister meets you in the hallway. Inside, the lights are off, the windows are covered with aluminum foil, and dishes are piled in the sink. Pt is standing in the far corner of the living room, pacing a short line, muttering, and glancing at the air vent. No weapons visible. No odor of alcohol. Pt is unkempt and appears to have lost weight. Clear path to the exit must be maintained. **Proctor note:** If the EMTs do not confirm the scene is safe, identify an exit, or ask the sister about weapons/violence history before entering, stop and discuss.
Pertinent Primary Assessment Findings	**AVPU:** Alert. A&O x3 - knows person, place (his apartment), and approximate date; does not have an accurate understanding of the event (believes the EMTs were sent by the neighbor). **A:** Patent. **B:** 18/min, regular, unlabored, lung sounds clear. **C:** Radial pulse 96, strong and regular. Skin pink, warm, dry. Cap refill <2 sec. No bleeding.

Pertinent Secondary Assessment Findings	**Mental status exam:** Appearance disheveled, poor hygiene. Behavior guarded, pacing, responding to internal stimuli (auditory hallucinations - two voices). Speech is coherent but tangential. Thought content: paranoid delusions (neighbor surveillance, medication is a tracking device). Denies visual hallucinations. **Denies suicidal ideation. Denies homicidal ideation** - although the voices "say the neighbor is dangerous," he has no plan to hurt anyone and wants to be left alone. **Physical:** No trauma. Pupils equal and reactive, 4 mm. No tremor, no rigidity, no fever. Mouth dry. No track marks. **SAMPLE** (patient gives partial answers; sister fills in) A - NKDA M - Risperidone 4 mg daily (has not taken in ~3 weeks). Sister has the bottle - it is nearly full. P - Schizophrenia, diagnosed at age 24. Two prior hospitalizations for the same thing after stopping meds. No history of violence. No other medical problems. Denies drug use; sister confirms, except occasional marijuana. L - Pt says "I don't eat their food." Sister thinks he has eaten very little for several days. Last water a few hours ago. E - Gradual decline over 3 weeks since stopping medication: isolating, covering windows, calling the sister at night about the neighbor. Today the sister found him talking to the vents and called 911.
Vitals	BP 132/84, HR 96 regular, RR 18, BGL 88 mg/dL, SpO2 98% RA, T 98.6 F, pupils PERRL 4 mm. **Set 2 (en route):** BP 128/82, HR 88, RR 16, SpO2 98% RA.
Treatments	**Necessary:** - Scene safety: request/confirm PD, identify exit, ask sister about weapons and prior violence, do not let the patient get between you and the door, keep 2 EMTs with one designated to talk. - Verbal de-escalation: introduce yourself, state honestly why you are there, speak slowly and quietly, use the pt's name, keep a non-threatening posture and distance (6+ ft initially), offer simple choices ("Would you rather sit on the couch or the stretcher?"). - Do not argue about delusions or hallucinations; do not play along either. Acknowledge the FEELING ("That sounds really frightening") without confirming the content. - Ask permission before every touch and explain each step ("I'd like to put this cuff on your arm - is that okay?"). - Full assessment with vitals and BGL to rule out medical causes of altered behavior (hypoglycemia, hypoxia, head injury, intoxication, infection, withdrawal). - Directly assess for suicidal and homicidal ideation. - Transport to an appropriate facility (ED with psychiatric capability). Continuous reassessment. Document mental status findings and quotes verbatim. **Helpful but not life-saving:** Allow the sister to ride along or follow if it helps the pt remain calm; bring the medication bottle; dim lights/turn off sirens in the ambulance; encourage oral fluids if pt will accept them. **Restraints:** Not indicated in this scenario. Discuss with the group: restraints are only used when the pt is an imminent danger to self or others, with enough personnel, per protocol, never prone, with continuous monitoring of airway and circulation.

Key Points	- Psychiatric patients deserve the same thorough medical assessment as anyone else. Never assume "it's just psych" - organic causes (hypoglycemia, hypoxia, drugs, head injury, sepsis, thyroid) can present identically. - Medication non-adherence is the most common cause of psychotic relapse. The near-full bottle is an important finding. - Hallucinations are real to the patient. Arguing worsens agitation; validating the feeling while staying honest is the goal. - Paranoid patients react poorly to being surrounded, surprised, whispered about, or touched without warning. - Scene safety and egress are the EMTs' first priority; PD staging is appropriate but the EMTs should not treat the pt as a criminal. - The pt is A&O x3 but his judgment is impaired by psychosis. If he refuses transport, the EMTs should consult medical direction/PD regarding an involuntary psychiatric hold per local protocol (e.g., CA WIC 5150) rather than simply accepting a refusal - gravely disabled (not eating, not caring for himself) may qualify. - Dehydration/poor intake for days is a real medical concern on top of the psychiatric one.
Bonus Questions	1. What is the difference between a hallucination and a delusion? (A: A hallucination is a sensory perception without a stimulus - hearing voices; a delusion is a fixed false belief - the neighbor has cameras in the vents.) 2. Name the "positive" and "negative" symptoms of schizophrenia. (A: Positive = hallucinations, delusions, disorganized speech/behavior. Negative = flat affect, social withdrawal, lack of motivation, poor hygiene.) 3. Which type of hallucination is most common in schizophrenia and which type suggests a medical or drug cause instead? (A: Auditory is most common in schizophrenia; visual or tactile hallucinations should raise suspicion for delirium, intoxication, withdrawal, or other organic cause.) 4. What class of drug is risperidone and what side effects might an EMT see? (A: Atypical antipsychotic; sedation, orthostatic hypotension, extrapyramidal symptoms/dystonia, and rarely neuroleptic malignant syndrome - fever, rigidity, altered mental status.) 5. What are the criteria for an involuntary psychiatric hold in your area? (A: Generally danger to self, danger to others, or gravely disabled as a result of a mental disorder - verify local protocol.) 6. If restraints became necessary, what position must be avoided and why? (A: Prone/hogtied - risk of positional asphyxia.)

47 YOM w/ Suicidal Ideation (Plan, Means, Note)

Scenario Set Up	**Equipment needed:** BP cuff, stethoscope, pulse oximeter, glucometer, blanket, stretcher. No wound care should be required (no injury in this scenario). **PROCTOR #1 (patient):** You are a 47-year-old male sitting on the edge of your bed in a dim bedroom. You lost your job six weeks ago, your wife moved out two weeks ago, and tonight you wrote a note to your two kids and called your brother to "say goodbye." Your brother called 911. You have NOT taken anything and have NOT hurt yourself. You are quiet, flat, and slow to answer; you avoid eye contact and give short answers. You are not angry and not aggressive. You are embarrassed that 911 was called. You will only disclose that you have a plan and have access to means if the EMTs ask you DIRECTLY and without judgment ("Are you thinking about killing yourself?" / "Do you have a plan?"). If they ask vaguely ("You're not thinking of doing anything silly, are you?") or sound uncomfortable, deny everything and say you just want to sleep. If they are direct, calm, and sit at your level, you open up and eventually agree to go with them. Initially you say you don't want to go and "you can't make me." **PROCTOR #2 (brother):** You arrived just before the ambulance. You can confirm the phone call and the note, and you know his medical history. You are scared and keep trying to answer for him - the EMTs should politely redirect you so they can talk to the pt directly. Tell the EMTs there is a firearm in the house if they ask about weapons (it is locked in a safe in the garage; the pt does not have it on him).
Dispatch	Respond Code 3 to a 47-year-old male, suicidal, conscious and breathing, caller is the patient's brother who is on scene. PD dispatched and en route.
Scene Size Up	Single-story house, brother meets you at the front door and says "he's in the back bedroom, he hasn't done anything yet." House is quiet and dark. Pt is sitting on the edge of the bed, hunched over, hands in his lap, a handwritten note on the nightstand. No blood, no pill bottles visible, no weapon visible, no odor of alcohol. Pt looks up briefly when you enter and looks back down. **Proctor note:** EMTs should ask about weapons in the home before/upon entry and should wait for or coordinate with PD per local protocol. They should NOT leave the pt alone at any point.
Pertinent Primary Assessment Findings	**AVPU:** Alert, A&O x4 (person, place, time, event). Responds slowly but appropriately. **A:** Patent. **B:** 16/min, regular, unlabored, lung sounds clear and equal. **C:** Radial pulse 78, strong, regular. Skin pink, warm, dry. Cap refill <2 sec. No bleeding, no visible injuries (EMTs should check wrists/forearms/neck during the exam - nothing found).

Pertinent Secondary Assessment Findings	**Mental status:** Appearance unkempt, 3-4 days of beard growth. Flat affect, slowed speech, long pauses, poor eye contact. Oriented, thoughts are organized, no hallucinations or delusions. Reports hopelessness ("nothing's going to get better"), guilt about the kids, not sleeping, no appetite, no interest in anything for weeks. **Suicide risk assessment (only if asked directly):** Admits to suicidal ideation for about two weeks, daily and increasing. Admits he has a specific plan. Admits he has access to the means for that plan. Wrote a note tonight. Called his brother to say goodbye. Denies ever attempting before. Denies homicidal ideation. Protective factor: says he "couldn't do it to the kids" - this is what stopped him tonight. **Physical exam:** No injuries. Pupils PERRL. Abdomen soft. No signs of ingestion. Ask the pt directly whether he has taken anything - he says no, and the brother confirms the pt was "fine" on the phone 30 minutes ago. **SAMPLE** A - Penicillin (rash) M - Lisinopril 10 mg daily. Was prescribed sertraline by his doctor 3 weeks ago but "never filled it." P - Hypertension. Depression diagnosed 3 weeks ago by his primary care doctor. No prior psychiatric hospitalization, no prior attempts. Brother reports the pt's father died by suicide when the pt was a teenager. Denies drug use; drinks 2-3 beers most nights, none tonight. L - Half a sandwich at lunch; no dinner. Coffee only. E - Job loss 6 weeks ago, wife moved out 2 weeks ago, progressive hopelessness and insomnia. Tonight he wrote a note and called his brother to say goodbye; brother called 911.
Vitals	BP 146/92, HR 78 regular, RR 16, BGL 102 mg/dL, SpO2 98% RA, T 98.2 F, pupils PERRL. **Set 2 (en route):** BP 142/90, HR 76, RR 16, SpO2 98% RA. Pt remains calm and cooperative.

Treatments	**Necessary:** - Scene safety: ask about weapons/medications in the home, coordinate with PD, keep an exit, never leave the pt alone (including in the bathroom or when getting dressed). - Establish rapport: sit down at or below the pt's eye level, introduce yourself, speak calmly, allow silence, redirect the brother so the pt can speak for himself. - Ask DIRECTLY and non-judgmentally about suicidal ideation, plan, means, intent, timeline, and prior attempts. Asking does not "plant the idea." - Full medical assessment and vitals including BGL and a search for injuries or evidence of ingestion. - Secure/remove means from the patient's immediate reach (have PD/family secure the firearm; collect any medications); the pt should not be allowed to retrieve items unsupervised. - Transport to an ED for psychiatric evaluation. The pt has ideation + plan + means + a note + a goodbye call = HIGH risk. This pt should not be left at home even if he "promises" to be fine. - If he refuses: calmly explain why he needs to be seen, involve PD/medical direction for an involuntary hold per local protocol (e.g., CA WIC 5150 - danger to self). Do not threaten or deceive; do not leave. - Continuous observation during transport; pt should be positioned so he cannot easily open doors or access equipment. Bring the note and medication list for the receiving facility. **Helpful but not life-saving:** Blanket, let him put on shoes with supervision, have the brother follow in his own vehicle, give a warm handoff to the ED staff including verbatim quotes.
Key Points	- Ask about suicide DIRECTLY. Vague or leading questions get denials. The patient in this scenario is scripted to disclose only to direct, calm questioning. - Risk factors present: male, middle-aged, recent job loss, recent separation, depression, alcohol use, family history of suicide, access to a firearm, and a specific plan with a note and goodbye calls. A sudden calm after a period of agitation can also be a warning sign (decision has been made). - A "low-energy" quiet patient can be the highest-risk patient. Do not confuse calm with safe. - Every suicidal patient still gets a full medical assessment - overdose is the most common method of attempt and may be hidden; always ask what, if anything, was taken. - Never leave the pt alone. Do not let the pt "go grab something" from another room unaccompanied. - The patient is A&O x4 and may demand to refuse. The EMTs should recognize that a patient who is an imminent danger to self generally cannot give a valid refusal, and should use PD/medical direction and local hold laws rather than walking away. - Document in the patient's own words. Do not use judgmental language ("committed suicide," "attention-seeking"). Use "died by suicide," "suicide attempt." - Debrief the student crew: these calls are emotionally heavy for providers too; mention peer support/EAP and the 988 Suicide & Crisis Lifeline as a resource for patients, families, and responders.

Bonus Questions	1. What questions make up a basic field suicide risk assessment? (A: Ideation - are you thinking of killing yourself? Plan - how? Means - do you have access? Intent/timeline - when? Prior attempts? Protective factors - what has stopped you so far?) 2. Does asking a patient directly about suicide increase their risk? (A: No. Research shows direct questioning does not increase risk and improves the chance of disclosure and help.) 3. List five risk factors present in this patient. (A: See Key Points - male, age, job loss, separation, depression, alcohol, family history, firearm access, plan + note.) 4. Why is the patient's family history relevant? (A: A family history of suicide is an independent risk factor.) 5. The patient is alert and oriented x4 - can he legally refuse transport? (A: Generally no; being an imminent danger to self impairs capacity for that decision. Use local involuntary hold procedures with PD and medical direction.) 6. The patient was recently started on an SSRI (sertraline). Why does that matter? (A: SSRIs can take 2-6 weeks to work, and there is a boxed warning about increased suicidal thinking in the first weeks of treatment, especially in younger patients - though this pt never started it. Also note any SSRI/other med ingestion risk.) 7. What is the 988 Lifeline? (A: The national Suicide & Crisis Lifeline - call or text 988, 24/7.)